Decision Advantage: partner sponsorship brief

A small, evidence-labelled first step: test whether one delayed-transfer-of-care escalation decision is governed end to end

Draft for internal EY discussion, 1 November 2026. Submitted by an EY member through EY's internal innovation process. The request is for sponsorship only, with no licence, equity or revenue share.

Evidence labels. *Verified*: confirmed in an official or primary source. *Plausible*: from a credible secondary source or a reasonable inference, but not confirmed. *Unsupported*: no source found. *Unknown*: not publicly documented. S and V numbers point to the source table on the last page. *Illustrative*: an example, not a forecast.

Executive summary

The problem. Irish hospitals can already see pressure in near real time. Every morning the HSE publishes 8am trolley counts and delayed transfers of care (DTOC) for each hospital, colour-coded against that hospital's threshold (Verified, S1). On 31 October 2026 national DTOC stood at 562 (Verified, S1), against a UEC Operational Plan KPI of 300 (Verified, S3). What happens after a breach is not published, and no public HSE source we found describes it: who owned the response, the deadline, why that action was chosen, whether it was possible, and whether it worked (Unknown, S1-S3).

The hypothesis. For one cross-team decision, a lightweight, read-only "decision contract" would show where breach responses stall. The contract records the trigger, accountable owner, deadline, escalation, rationale, feasibility class (decision delayed / action infeasible / action done without expected result) and verified outcome. It sits after the insight. It does not replace hubs, command centres, EHRs or digital twins. Humans make the decisions; the method only records and tests them.

Provisional lead use case: DTOC escalation after a site threshold breach. It tied with ED surge activation at 34/45; specialist clinic capacity scored 30/45. DTOC wins the tie for four reasons. The decision crosses acute and community teams, and an Irish review found that accountability between them was contested (Verified, S19; 2018). The National Service Plan 2026 prioritises it (Verified, S2). Reasons for delay are already reported in aggregate categories (Verified, S4). And patients in scope have already been judged ready for transfer, which keeps the method non-clinical.

What weakens the case. Capacity is the main driver: on 6 July 2026, 217 of 526 delayed transfers were waiting for residential care (Verified, S4). Published evidence that escalation or decision delays add to DTOC is weak and mostly qualitative (section 3b). Parts of the contract already exist elsewhere. The HSE Patient Flow Academy template records action owners, target dates and delay reasons for long-stay patients (Verified, S8). LeanTaaS documents action owners and closed-loop outcome tracking (Verified, V4). Nervecentre, the HSE's national Acute Floor Information System supplier (Verified, S17), documents named accountability and delay reasons in an NHS deployment (Verified, V15). That leaves one narrow candidate gap: an aggregate, cross-organisational record of the breach response, with its feasibility class and an outcome check. No public source documents such a record, but that does not prove it is missing.

Recommended commercial model: (iii) hybrid. Lead with an advisory diagnostic. Use a narrow DTOC workflow prototype only in shadow mode, on aggregate data. Productise only after repeat evidence at three or more sites. Software-first is not recommended: HIHI judged the market crowded (Verified, S5), and the earlier licence figures are unvalidated planning assumptions (EUR 75,000-250,000 per site; SAM EUR 4.35m, which equals 29 sites x EUR 150,000).

The ask. Step 1: sponsor a 90-minute internal workshop. If EY has appropriate relationships, also make warm introductions to HSE patient-flow or UEC contacts; no such relationship was verified in public sources. Step 2, only if Step 1 passes: a 12-week assessment with [__] partner hours and [__] team members.

Not yet validated:

- that the gap exists at any real site
- that hubs do not already record owners and outcomes
- a buyer and budget
- a data route that needs no patient-level data
- that a prototype adds value over a simple template
- EY's internal route and the IP and independence position

1. Use-case discovery and selection

1a. Longlist. The longlist combines the three seed cases with candidates where public HSE sources show a funded priority. Screening rule: a threshold or model signal exists, the decision that follows crosses teams, and it can be tested without patient-level data.

#	Candidate: signal and decision	Public HSE anchor	Screen
1	**ED surge activation:** when the 8am trolley count breaches the site threshold, decide whether to open surge beds or invoke the escalation plan or Full Capacity Protocol	UEC report thresholds (S1); NSP 2026 UEC actions 1 and 7 (S2); trolley KPI 280 (S3)	Shortlist
2	**DTOC escalation:** when site DTOC breaches its threshold, escalate across acute and community services (transitional care, community beds, home support, rehab)	NSP 2026 DTOC thresholds and Regional Operational Plans (S2); DTOC KPI 300 (S3)	Shortlist
3	**Specialist clinic capacity (oHCM-type):** when a planning model forecasts demand above capacity, add sessions, arrange cross-cover or record that no action is feasible	NSP 2026: OPD clinic planning tool in all hospitals, +103,000 appointments (S2); EY article (S15)	Shortlist
4	Elective cancellation while surge is in use: cancel or protect planned admissions	UEC report: surge may cancel planned admissions (S1); Planned Care Wait Time Action Plan 2026 (S2)	Merged into 1 (it is a surge action)
5	Transitional care or community bed release: approve funding or allocate a community nursing unit (CNU) bed	NSP 2026: transitional care for 10,800 people; National Bed Management System for CNUs (S2)	Merged into 2 as a permitted action; overlaps with the planned CNU system
6	Long-stay (>14 days) review escalation: weekly multidisciplinary team actions	Patient Flow Academy LOS guideline (S8)	Dropped: patient-level; owner, date and reason fields already specified
7	Virtual ward use under acute pressure: step patients down to virtual beds	Minister on unused virtual beds (S4); virtual ward expansion (S18)	Dropped: patient selection is clinical; no published threshold

1b-c. Shortlist scoring (1 = weak, 5 = strong). E = evidence; D = what would disprove the score.

Criterion	ED surge activation	DTOC escalation	Specialist clinic capacity
Real, measurable problem	**5** E: 187 trolleys nationally on 31 Oct 2026, published daily against site thresholds (S1). D: breaches are rare or clear within hours.	**5** E: 562 DTOC on 31 Oct 2026 vs KPI 300 (S1, S3); a TD reported 86 consecutive days above 500 (Plausible, S4). D: DTOC stays below site thresholds for a quarter.	**3** E: NSP funds a clinic tool to add 103,000 appointments (S2); the oHCM site and figures are not public (S15). D: no clinic forecasts demand above capacity.
Alignment with HSE plans and funding	**5** E: NSP 2026 UEC actions 1 and 7: escalation protocols, 8am thresholds, recovery plans, hubs (S2). D: the 2026-27 UEC plan drops thresholds.	**5** E: NSP 2026: DTOC thresholds; Regional Operational Plans with a strong DTOC emphasis; transitional care funding (S2). D: as for surge.	**4** E: NSP 2026: OPD clinic planning tool in all hospitals (S2). D: the roll-out already includes decision follow-up.
Existing thresholds and decision owners	**4** E: site thresholds (S1); an accountable officer per level in the 2023 performance framework (S7). D: local plans name no owner per action.	**3** E: site DTOC thresholds (S1, S2); the 2018 review found accountability contested (S19). D: the Health Regions now name one owner per site.	**2** E: no published clinic-capacity threshold or owner found. D: the clinic tool defines both.
Clear buyer, budget holder and funding route	**3** E: Regional UEC Improvement Programmes (S2); no budget line for decision governance found. D: no regional lead holds discretionary spend.	**3** E: Regional UEC programmes and IHAs lead DTOC plans (S2); no budget line found. D: as for surge.	**3** E: planned-care programme; EY reportedly supports the clinic tool (Plausible, S13). D: that programme cannot take add-ons.
Data accessible without patient-level data	**4** E: aggregate trolley and surge counts are published daily (S1). D: escalation actions are recorded only in patient records.	**4** E: aggregate DTOC by site (S1) and by reason category (S4). D: reason categories exist only per patient at site level.	**3** E: needs clinic templates and aggregate waiting-list counts; availability not verified. D: only patient-level lists exist.
Non-clinical boundary	**3** E: surge actions (ward boarding, cancellations) carry clinical risk. D: clinical governance classes surge decisions as clinical.	**4** E: DTOC patients have already been judged ready for transfer (S19); escalation is about placement, funding and support. D: escalations routinely need clinical re-review (90 'very complex clinical needs' cases, S4).	**3** E: sessions are operational, but therapy eligibility and prioritisation are clinical (S15). D: the two cannot be separated in practice.
Fit with EY's public work and offerings	**2** E: no public EY urgent-care work found. D: a public EY UEC engagement exists.	**2** E: no public EY DTOC work; the NSCR is adjacent because it holds discharge summaries (S11). D: as for surge.	**5** E: EY's oHCM digital twin article (S15); reported support for the clinic tool (Plausible, S13). D: the Tenderwatch record is wrong.
Low-friction EY adoption	**3** E: Step 1 needs only process-advisory skills and a synthetic scenario. D: no partner can name an ED or UEC contact.	**3** E: as for surge, but Step 2 needs access to an IHA and a hospital. D: no partner has a relationship with any region or IHA.	**4** E: builds on a published EY approach and its authors. D: independence or scope rules block any link to live HSE work.
Scalability across HSE regions	**5** E: the same 8am measures at 29 sites in six Health Regions (S1, S2). D: local escalation plans are too varied to template.	**5** E: the same national measures; DTOC is also a documented problem in the Netherlands and UK (S20, S21). D: as for surge.	**3** E: could generalise through the clinic tool, but oHCM itself is niche. D: no second specialty shows the pattern.
Total (of 45)	**34**	**34**	**30**

1d. Recommendation: DTOC escalation (provisional). Surge and DTOC tie. The tie-break rule is to prefer stronger public evidence and more reachable stakeholders, and on both counts DTOC is ahead:

- Irish evidence speaks directly to unclear accountability and late recording of discharge decisions (S19).
- The Minister has publicly questioned how available step-down capacity is used (S4).
- The national AFIS incumbent is focused on the acute floor rather than acute-to-community escalation (Plausible, S17).
- Discharge coordinators and IHA community managers are a defined group that can be reached.

Clinic capacity has the best EY fit but the weakest public evidence of thresholds and owners. **Interview finding that would change the choice:** staff might show that red-DTOC escalations already carry a named owner, deadline and recorded outcome (in hub logs, regional UEC reporting, Nervecentre or the planned CNU system). Or nearly every breach might turn out to be a pure supply shortfall with no decision pending. In either case, switch to surge activation, but only if ED managers confirm that escalation decisions go unrecorded. Otherwise, stop.

2. Gap map for DTOC escalation (three capabilities only)

Public documentation was reviewed in October and November 2026, using vendor product documentation in preference to marketing where available. 'Not documented' means the reviewed public sources do not describe the capability, not that the product lacks it. No deficiency is claimed.

Vendor / practice	Decision rights and ownership	Rationale capture	Outcome verification	What to concede
GE HealthCare Command Center	**Not documented.** Tiles alert and prioritise; 'tile owners' write the specifications and are not action owners NIHR study	**Not documented.** Operational notes and barrier huddles; no recorded 'why' per action exec brief	**Not documented.** Aggregate case-study results only outcomes	Real-time integration, forecasting, escalation management; NHS Bradford contract (S26)
LeanTaaS iQueue	**Documented.** Huddle Assistant defines actions 'with clear ownership' blog	**Not documented.** Captures decisions; no reason codes described inpatient flow	**Documented.** Tracks whether actions were completed and expected outcomes achieved blog	Closest to the full contract; no UK or Irish deployment found
Qventus	**Documented.** A model-triggered nudge goes to a named role, who accepts or declines Allina	**Not documented.** Accept or decline with no recorded reason Allina	**Not documented.** KPI dashboards only OR utilisation	Model-triggered nudges and role-based dashboards
TeleTracking	**Documented (role level).** Assigned transport and cleaning jobs UK apps	**Not documented** Decision IQ	**Not documented.** Dashboards for analysis and action planning UK analytics	System-wide capacity visibility, forecasting, simulation
Epic	**Documented.** Transport and cleaning requests are auto-assigned patient flow	**Not documented** patient flow	**Not documented.** Aggregate pilot results and weekly reviews Sutter	Real-time capacity; predicted census and length of stay
Oracle Health	**Documented.** Automated job assignment patient flow	**Documented (narrow).** Transfer Center records admit and decline decisions with reasons systems ops	**Not documented** systems ops	Near-real-time capacity and predictive flow
Nervecentre	**Documented.** Discharge cohorts are assigned to a named individual: 'who's accountable' Nottingham	**Documented (partial).** Records why discharge delays occur Nottingham	**Not documented.** Timers and escalation prompts, but no check of an action's effect NUH NeT	HSE national AFIS supplier, 40 hospitals HSE news; strongest local incumbent
HSE current practice (hubs, guidance)	**Documented in guidance (patient level):** action owner, target date, escalation flag LOS guideline; an accountable officer per performance-escalation level 2023 report. Hubs: **Unknown**	**Documented in guidance (patient level):** reason and cause category LOS guideline; aggregate DTOC reasons are reported PQ Jul 2026. Hubs: **Unknown**	**Not documented at action level:** KPI and RAG reporting only PQ Feb 2026. Hubs: **Unknown**	The HSE already has the vocabulary; reuse it rather than duplicate it

What to concede. Several elements are already documented elsewhere:

- ownership and task assignment, by most vendors;
- closed-loop outcome checks, by LeanTaaS;
- reason capture in narrower settings, by Oracle and Nervecentre;
- owners, dates and reasons at patient level, in the HSE's own guidance.

Decision Advantage therefore cannot claim novelty for any single field. **The remaining hypothesis to test:** no reviewed source documents an aggregate record of the acute-to-community response to a DTOC threshold breach that classifies the response (decision delayed / action infeasible / action done without expected result) and checks the result. An incumbent (Nervecentre, HPVP or the planned CNU system; S2, S18) could plausibly add this, so HIHI's concern about differentiation still stands (S5).

3. Verification of key facts

Claim	Status	Source
EY, with Better and Kainos, was awarded the contract to build the National Shared Care Record (NSCR) technology platform; contract in Q1 2025; read-only	Verified	S9
Contract value of EUR 81m to an EY-led consortium	Plausible (press; award notice not checked)	S10
The NSCR sits within the HSE's MyHealth@IE programme; HSE and EY speakers presented it in September 2026	Plausible (secondary)	S12
Any EY role in MyHealth@IE beyond the NSCR	Unsupported (none found)	-
EY won HSE mini-competitions in March 2026 for 'support for the HSE implementation of the Outpatient Department Clinic Planning Tool' and for diagnostics and primary care consultancy; 13 HSE awards in 2023-26	Plausible (aggregator; verify on eTenders)	S13
The UEC Report publishes, per hospital: 8am trolleys (ED and wards), waits over 24 hours, over-75s waiting over 24 hours, DTOC (as of midnight) and surge capacity in use (2pm), colour-coded against hospital thresholds; a breach triggers a hospital response, usually surge capacity. It does not publish who decided, why, recovery-plan actions or outcomes	Verified	S1
The Shared Care Record is a read-only, patient-level clinical record: medications, GP lab and radiology reports, referrals, discharge summaries, appointments, waiting lists, HIPE procedures, vaccinations. Authorised staff on the HSE network can access it after e-learning; ADT and ED events are planned for 2026	Verified	S11
It is therefore not an operational flow data source for this method	Plausible (inference)	S11
Each of the 29 sites is to have green/amber/red 8am thresholds for trolleys and DTOC, with recovery plans; centralised hubs in all hospitals	Verified (plan commitment)	S2
KPIs: average 8am trolleys 280 and DTOC 300; daily situational analysis; quarterly RAG reports by Health Regions	Verified	S3
Content of site recovery plans	Unknown (not published)	-
Whether hubs record decision owner, rationale or outcome	Unknown (not documented)	S2, S3
The Full Capacity Protocol 'appears rarely used'; persistent breaches get no routine formal independent review (2023)	Verified (report content)	S7

3b. Root cause check (3 searches). Evidence that capacity dominates is strong:

- Of 526 DTOC on 6 July 2026, 217 were waiting for residential care, 90 had very complex clinical needs, 68 involved legal complexity, 61 rehab access, 42 home support, 35 housing and 13 non-compliance (Verified, S4).
- In three Dutch hospitals, 74% of discharge delays had causes outside the hospital (Verified, S20).

- The UK National Audit Office (NAO) cited workforce capacity as a significant cause (Verified, S21).

Evidence for decision or escalation delay is thinner:

- Ireland's 2018 expert review found that stakeholders disagreed on who is accountable and that discharge decisions were recorded late; it also found that late capture of decision data limits early intervention (Verified, S19).
- In the Dutch study, the most frequent in-hospital reason was waiting for a physician's decision or review. That is a clinical decision and outside scope (Verified, S20).
- The NAO found no incentive for community bodies to receive patients faster (Verified, S21).
- The Minister said some patients could go to virtual beds that are not being used (Verified as a statement, S4).

Verdict: weak. No study we found isolates or quantifies escalation delay after a DTOC threshold breach, and the Irish review predates the Health Regions. Step 2 must test this first. If decisions are rarely the constraint, the method's value shrinks to making infeasibility visible for capacity planning.

4. Commercial model comparison

No fees are estimated. Buyers and funding lines are those visible in public plans; none has been tested with the HSE.

	(i) Advisory: diagnostic, then pilot, then repeatable service	(ii) Productised software / SaaS	(iii) Hybrid: advisory-led, shadow-mode prototype, productise later
Buyer and funding line	Regional UEC Improvement Programme or IHA manager; NSP 2026 UEC and Regional Operational Plan priorities (S2)	HSE Technology & Transformation; NSP 2026 digital priorities, including the National Innovation Pathway (S2)	As (i) at first; Technology & Transformation only if productised
Procurement route	HSE business and management consultancy, 'regularly conducted via the OGP framework' (Verified, S14), through mini-competition	Competitive tender, or an innovation-ready procurement model (still being defined, S2); prior-involvement rules apply if EY advised (S23)	Advisory route first; any software through a separate, later competition
Upfront investment and capabilities (indicative)	Partner and manager time; a patient-flow subject expert; templates; no build	Product build, security, integration, hosting, support; IP licence from the innovator (verify); medical-device qualification check (S22)	As (i), plus re-scoping the synthetic prototype to one DTOC workflow on aggregate data
Time to first evidence	About 12 weeks (Step 2)	Longest: build, information governance and procurement come before any live evidence	Weeks 8-10 of Step 2 (shadow mode)
Key risks	Easy to copy; the HSE Patient Flow Academy could run it in-house (S8)	Crowded market (S5); incumbents could add the feature (V17, S18); unvalidated price; vendor-independence questions	The prototype still needs security and DPIA screening; scope creep towards software; IP
Scalability (HSE and beyond)	Tied to people's time; templates are reusable	Highest in theory, but each site needs integration	Template, rules library and metrics are reusable; product decision after three or more sites
Fit with EY's public offerings	EY Ireland health advisory on operating models and analytics (S16)	No public EY patient-flow product found	Mirrors the lesson of EY's twin article: anchor on one decision (S15)

Price benchmarks. The HIHI submission's licence (EUR 75,000-250,000 per site a year, base EUR 150,000) is unvalidated. Its figures are ceilings, not forecasts (arithmetic on S5):

- the SAM of EUR 4.35m equals 29 sites x EUR 150,000, i.e. every UEC-report site buying at the base price;
- the TAM of EUR 8.4m equals 56 sites at the base price.

The only comparable public price found is GBP 803,375 for three years' maintenance and support of Bradford's GE Command Centre Tiles and video wall (about GBP 268,000 a year; Verified, S26). That covers a mature, hospital-wide command centre, not a single-decision tool. The NSCR national platform (EUR 81m; Plausible, S10) is a scale reference only. No public price was found for Nervecentre's AFIS, HPVP, LeanTaaS, Qventus, TeleTracking or Epic patient flow. A single-decision tool priced near the EUR 150,000 base would compete for the same budget as the national AFIS and HPVP, so the price must be tested, not assumed.

Value logic (no savings claimed). Measure in this order:

- the time from breach to a recorded decision;
- the share of breach responses with an owner, a rationale and a verified outcome;
- the mix of feasibility classes. An 'action infeasible' record is evidence for capacity planning, for example for the forthcoming residential care capacity plan (S4);
- only then, any change in DTOC days against a baseline.

A TD cited about EUR 1,300 a day for an acute bed (Plausible, S4), but no saving is claimed.

Recommendation: option (iii). It gives the earliest evidence on whether tooling adds anything over a template, with no software investment or software procurement. **What would change it:**

- HSE information governance or EY Risk Management rules out any tooling in an assessment: move to option (i);
- three or more sites show the same gap and incumbents cannot configure a fix: consider option (ii), possibly with an incumbent;
- the interviews disprove the gap: stop.

5. Scalability

From one hospital to six Health Regions. Start with one hospital and its IHA community services, then the Health Region, then the other regions. All 29 sites publish the same 8am measures (S1, S2).

Reusable: the decision-contract template; a rules library mapping DTOC reason categories to permitted actions (transitional care funding, home support, community beds, rehab; S2, S4); metrics (time to decision, feasibility mix, verified-outcome rate); the interview script; a DPIA screening template.

Bespoke: local thresholds, named roles, escalation routes and data extracts. A repeatable service should require proof at three sites in at least two regions (Illustrative threshold).

EU. Delayed discharge is documented outside Ireland. In three Dutch hospitals, 21% of surveyed inpatients no longer needed acute care (Verified, S20). In England, outside the EU, the NAO put the gross annual cost at about GBP 820m (Verified, S21; 2016). The method is portable because it records a decision rather than integrating a product. Three EU frameworks shape scaling:

- EU public-procurement rules apply in every Member State, including the prior-involvement rule (S23).
- The European Health Data Space entered into force in March 2025, but its secondary-use rules apply only from March 2029 (Verified, S24), so it does not yet ease data access.
- MDCG 2019-11 states that software for non-medical purposes, such as staff planning, is not medical device software (Verified, S22), so the intended purpose must stay operational.

Any EU step should follow repeat Irish evidence, because local escalation frameworks differ by country.

6. Fit with EY's public work

EY Ireland's article on digital twins of patient journeys (7 September 2026; Dr Mary Coghlan, Partner, AI & Data; Mark Chalmers, Partner, Life Sciences) describes an oHCM clinic twin. It concludes that twins are most valuable when anchored in a specific issue or decision, and that clinical judgement remains essential (Verified, S15).

Decision Advantage is complementary. A twin tests options before a decision; the contract records the decision actually taken and whether it worked. Comparing recorded outcomes with a twin's assumptions is a hypothesis, not a claim.

EY's work on the National Shared Care Record (Verified, S9) is a clinical record for direct care, and this method neither uses it nor depends on it. EY Ireland's health page describes work on operating models, analytics and Smart Health Solutions (Verified, S16). Nothing here asserts EY's internal strategy or that EY will adopt the idea.

7. Validation plan

Six to eight 45-minute interviews, roles only:

- HSE Technology & Transformation (UEC data or HPVP lead)
- Regional UEC Improvement Programme lead
- hospital patient-flow or bed manager
- acute discharge coordinator
- IHA older persons or community services manager
- ED clinical nurse manager
- data protection officer
- optional: a Patient Flow Academy member

Ground rules: no pitching, only synthetic or anonymised examples, and no patient details.

Script (Mom Test: past behaviour and current workarounds; no hypotheticals; no leading wording)

1. Tell me about the last time your site's DTOC count went red at 8am. What happened next?
2. Who did what, and how did each person find out?
3. Where was the response written down, if anywhere? Could you show me a blank or anonymised example?
4. How many red-DTOC days did you have last month, and where does that number come from?
5. The last time an escalation did not bring the count down, how did you find out, and what did you do?
6. What do you currently use to track escalations into community services: a system, a spreadsheet, email or huddle notes?
7. How much time did that tracking take last week, and who did it?
8. **(Disproof)** When an escalation was last reviewed afterwards, what did the review show about who decided and why?
9. **(Disproof)** Of the delayed transfers on today's list, how many are waiting for someone's decision, and how many for a bed, a care package or a legal process?
10. **(Disproof)** Has anyone tried to fix escalation follow-up before, for example with a hub log, Nervecentre, HPVP or a spreadsheet? What happened?

Go to Step 2 only if all of these hold:

- at least four interviewees, from at least two sites, independently describe a recent breach where the owner, rationale or outcome was not recorded;
- one site or IHA lead agrees to explore an aggregate data extract;
- the data protection officer sees a route that needs no patient-level data.

Stop if any of these holds:

- two or more sites show an existing record that meets the contract (Q8, Q10);
- most delays on recent days were supply waits with no decision pending (Q9);
- no budget holder emerges;
- the work would need patient-level processing.

These thresholds are illustrative and should be agreed in Step 1.

8. The ask

Step 1: sponsor now. Endorse a 90-minute internal workshop (agenda adapted from the earlier draft, S6):

- scope and stop conditions (10 minutes)
- a synthetic DTOC breach scenario (15)
- map presumed current practice (15)
- complete the decision-contract template (20)
- red-team duplication, capacity and burden (15)
- score and decide: stop or assess (15)

Attendees: health operations, AI & Data, and risk and independence [names to confirm]. About two hours of preparation; synthetic or published data only.

Warm introductions, if appropriate. Public sources show EY working with HSE Technology & Transformation on the NSCR (Verified, S9) and reportedly on the OPD clinic tool (Plausible, S13). No public EY relationship with HSE patient-flow, UEC or DTOC teams was found (Unknown; verify). Clear any introduction with the relevant engagement partner and EY Risk Management first.

Step 2: only if Step 1 passes. A 12-week assessment in four phases:

- diagnose (weeks 1-3): interviews, a workflow map, a review of existing tools and a falsifiable gap statement;
- design (weeks 4-7): the contract, controls, measures and DPIA screening;
- shadow mode (weeks 8-10): read-only, aggregate data, no action taken;

- decide (weeks 11-12): an evidence pack and a stop/go decision.

Resourcing: [__] partner hours and [__] team members. Gates before week 1: a named sponsor, a client route, resource approval, and IP and independence clearance.

How this leads into the model: Step 2 is the diagnostic and shadow-mode phase of option (iii). Its result decides whether a repeatable service exists and whether tooling adds anything over a template.

9. Adoption route inside EY (items to verify; none asserted)

- **Intake:** which EY Ireland channel takes a member's innovation idea, what it requires and who reviews it.
- **Sponsor:** whether the sponsor must be a partner in health, AI & Data or another practice, and what the sponsor commits to.
- **Approvals:** practice leadership, Risk Management and quality review, plus an independence check on HSE-related relationships before any external contact.
- **IP:** ownership and permitted use of the prototype (built as a personal project), of earlier submissions (HIHI) and of anything produced in Step 2. Ownership is not assumed.
- **If software follows:** IP assignment or licensing, security assessment, hosting, liability, software quality, and procurement eligibility given prior involvement (S23).

Minimum for a yes to Step 1:

- confirmation that the idea can be discussed internally
- one 90-minute slot
- about two hours of preparation
- synthetic or published data only
- no client contact before the independence check

10. Other risks

- **Data access and DPIA.** Use only aggregate site counts and reason categories. Small counts at small sites can still identify people, so screen the approach with the HSE data protection officer. GDPR Article 35 requires a DPIA where processing is likely to be high risk (Verified, S25). Any need for patient-level data stops the work. Data route: daily site counts are public (S1); reason categories exist nationally (S4); site extracts need HSE agreement (Unknown).
- **Non-clinical boundary.** Patient selection, clinical review and discharge decisions are out of scope. Keep the intended purpose operational so the method stays outside medical-device rules (S22). The 90 'very complex clinical needs' cases (S4) show where the boundary will be tested.
- **EY as an HSE vendor.** EY is building the NSCR (S9) and reportedly holds other HSE mandates (S13). The risks are perceived self-interest, conflicts, and prior involvement if this work shapes a later tender (S23). To mitigate, keep any Step 2 separately scoped, disclosed and independent of existing engagements.
- **Duplication and burden.** Incumbents may add the capability (V17, S18). Extra recording may burden busy staff, so measure that burden in shadow mode.

Source and verification table. Status reflects what each source supports as used in this brief. Vendor rows (V) were checked by documentation review in Oct-Nov 2026. Figures derived by arithmetic (56 sites, GBP 268,000 a year, 29 x EUR 150,000) are computed from the cited source.

ID	Source	Used for (figures and claims)	Status	URL
S1	HSE Urgent and Emergency Care daily report	What is published; 8am trolleys, DTOC (midnight), surge (2pm) vs hospital thresholds; breach triggers a response, usually surge; 31 Oct 2026: 187 trolleys, 562 DTOC	Verified	https://www2.hse.ie/services/urgent-emergency-care-report/
S2	HSE National Service Plan 2026	29 sites with 8am trolley and DTOC thresholds and recovery plans; hubs; Regional Operational Plans (DTOC); transitional care for 10,800; CNU bed system; OPD clinic tool +103,000; innovation pathway; six Health Regions	Verified	https://about.hse.ie/publications/hse-national-service-plan-2026/
S3	Oireachtas written answer, 10 Feb 2026	KPIs: 8am trolleys 280, DTOC 300; daily situational analysis; quarterly RAG reports; hubs; HPVP	Verified	https://www.oireachtas.ie/en/debates/question/2026-02-10/1047/
S4	Oireachtas oral answer, 9 Jul 2026	Minister: 526 DTOC on 6 Jul (217 residential, 90 complex clinical, 68 legal, 61 rehab, 42 home support, 35 housing, 13 non-compliance); unused virtual beds; residential capacity plan. TD: 86 days >500; ~EUR 1,300/day	Verified (Minister); Plausible (TD figures)	https://www.oireachtas.ie/en/debates/question/2026-07-09/94/
S5	HIHI Idea Assessment Report (attached; exported 7 Oct 2026)	Crowded market; differentiation; EUR 75k-250k licence, base 150k; TAM 8.4m; SAM 4.35m	Verified (document); figures unvalidated	Attached file: IdeaAssessment-HIHI.pdf
S6	Draft partner sponsorship proposal (attached)	Reused: workshop agenda, 12-week phases, stop criteria	Verified (document)	Attached file: Decision_Advantage_Partner_Sponsorship_Proposal_merged.docx
S7	ED Escalation Working Group framework report, Nov 2023	Full Capacity Protocol rarely used; no routine formal review; accountable officer per escalation level	Verified (content)	https://emnow.ie/wp-content/uploads/2025/06/ED-Escalation-Working-Group-Framework-Report-FINAL-November-2023.pdf
S8	HSE Patient Flow Academy, LOS Optimisation Guideline	Long-stay template: action owner, target date, reason, cause category, escalation flag	Verified	https://healthservice.hse.ie/documents/8039/HSE_Patient_Flow_Academy_Length_of_Stay_Optimisation_Guideline.pdf
S9	Oireachtas written answer, 2 Oct 2025	NSCR platform contract: EY, Better, Kainos; Q1 2025; read-only	Verified	https://www.oireachtas.ie/en/debates/question/2025-10-02/416/
S10	Irish Independent, 22 Sep 2025	EUR 81m contract to EY-led consortium	Plausible (press)	https://www.independent.ie/business/technology/ey-led-group-wins-81m-hse-patient-records-contract/a/132523779.html
S11	HSE Shared Care Record page	Contents, access rules, read-only, 2026 releases	Verified	https://about.hse.ie/our-work/technology/data-and-information-sharing/shared-care-record/
S12	openEHR blog, EHRCON26 reflections	NSCR within MyHealth@IE; HSE and EY presenters	Plausible (secondary)	https://openehr.org/chill-delightful-revelational-reflections-from-ehrcon26/
S13	Tenderwatch: HSE contract awards	EY: 13 HSE awards 2023-26; Mar 2026 OPD Clinic Planning Tool support; diagnostics and primary care consultancy	Plausible (aggregator)	https://tenderwatch.ie/awards/by-authority/health-service-executive-hse
S14	HSE procurement pipeline Q4 2025/2026	Consultancy 'regularly conducted via the OGP framework'	Verified	https://healthservice.hse.ie/documents/8095/HSE_Procurement_-_Pipeline_of_Upcoming_Opportunities_for_Suppliers.pdf
S15	EY Ireland: digital twins of patient journeys (7 Sep 2026)	oHCM clinic twin; authors; anchor on a decision; clinical judgement	Verified	https://www.ey.com/en_ie/insights/life-sciences/how-digital-twins-of-patient-journeys-can-help-improve-access-to-innovative-treatments
S16	EY Ireland Health page	Operating models, analytics, Smart Health Solutions	Verified	https://www.ey.com/en_ie/industries/health
S17	Digital Health News, 3 Oct 2023	AFIS: Cardiac Services with Nervecentre, 40 hospitals, acute floor scope	Verified (press)	https://www.digitalhealth.net/2023/10/nervecentre-chosen-for-ireland-wide-emergency-department-deal/
S18	HTN, 2 Sep 2025 (on Slaintecare & PfG 2025+)	HPVP scope and roll-out; virtual ward expansion	Plausible (secondary)	https://htn.co.uk/2025/09/02/irish-governments-focuses-for-next-phase-of-healthcare-transformation-national-shared-care-record-virtual-wards-ai-and-intelligent-automation-and-more/
S19	Independent Expert Review of Delayed Discharges, Nov 2018	Contested accountability; late recording of decisions; DTOC definition	Verified	https://assets.gov.ie/static/documents/independent-expert-review-of-delayed-discharges-november-2018.pdf
S20	van den Ende et al., Int J Qual Health Care, 2023	21% no acute need; 74% of delays outside hospital; 14% awaiting physician	Verified	https://academic.oup.com/intqhc/article/35/2/mzad022/7156525
S21	UK NAO, Discharging older patients from hospital, 2016	GBP 820m gross annual cost; workforce; incentives	Verified	https://www.nao.org.uk/reports/discharging-older-patients-from-hospital/
S22	MDCG 2019-11 software qualification guidance	Non-medical-purpose software is not medical device software	Verified	https://health.ec.europa.eu/system/files/2020-09/md_mdcg_2019_11_guidance_qualification_classification_software_en_0.pdf
S23	Directive 2014/24/EU, Article 41	Prior involvement of candidates	Verified	https://eur-lex.europa.eu/legal-content/EN/TXT/?uri=CELEX:32014L0024
S24	European Commission: EHDS Regulation	In force 26 Mar 2025; secondary use from Mar 2029	Verified	https://health.ec.europa.eu/ehealth-digital-health-and-care/european-health-data-space-regulation-ehds_en
S25	GDPR Article 35	DPIA where high risk	Verified	https://gdpr-info.eu/art-35-gdpr/
S26	UK Contracts Finder: Bradford award to GE	GBP 803,375; 3-year maintenance and support of Command Centre Tiles and video wall; May 2026	Verified	https://www.contractsfinder.service.gov.uk/Notice/0d76f5f3-b7ae-4277-bfd2-701eb1c94b69
V1	GE: NIHR HSDR implementation study	Ownership: Not documented	Verified (doc review)	https://www.journalslibrary.nihr.ac.uk/hsdr/TATM3277
V2	GE: Command Center executive brief	Rationale: Not documented	Verified (doc review)	https://s7d9.scene7.com/is/content/gehealthcare/Why%20GEHC%20Command%20Center%20Executive%20Brief%20_RGB-JB29667XX%20-1pdf
V3	GE: Command Center outcomes	Outcome: Not documented	Verified (doc review)	https://www.gehealthcare.com/en-us/products/software/command-center/outcomes
V4	LeanTaaS: smarter bed huddles	Ownership and outcome: Documented	Verified (doc review)	https://leantaas.com/blog/smarter-bed-huddles/
V5	LeanTaaS: iQueue Inpatient Flow	Rationale: Not documented	Verified (doc review)	https://leantaas.com/products/inpatient-flow/
V6	Qventus: Allina Health story	Ownership: Documented; rationale: Not documented	Verified (doc review)	https://www.qventus.com/resources/customer-successes/allina-health/
V7	Qventus: OR utilization	Outcome: Not documented	Verified (doc review)	https://www.qventus.com/solutions/operating-room-utilization/
V8	TeleTracking: UK mobile apps data sheet	Ownership: Documented (role level)	Verified (doc review)	https://eadn-wc03-11006557.nxedge.io/wp-content/uploads/2025/01/UK_Mobile_Apps_DataSheet.pdf
V9	TeleTracking: Decision IQ data sheet	Rationale: Not documented	Verified (doc review)	https://eadn-wc03-11006557.nxedge.io/wp-content/uploads/2026/02/Decision-IQ-Datasheet-1.pdf
V10	TeleTracking: UK data and analytics	Outcome: Not documented	Verified (doc review)	https://teletracking.uk/healthcare-operations-cloud/data-analytics/
V11	Epic: hospital patient flow	Ownership: Documented; rationale: Not documented	Verified (doc review)	https://www.epic.com/software/hospital-patient-flow/
V12	Epic: Sutter capacity management	Outcome: Not documented	Verified (doc review)	https://www.epicshare.org/share-and-learn/sutter-centralized-capacity-management
V13	Oracle Health: patient flow	Ownership: Documented	Verified (doc review)	https://www.oracle.com/il-en/health/clinical-financial-operations/
V14	Oracle Health: systems operations	Rationale: Documented (narrow); outcome: Not documented	Verified (doc review)	https://www.oracle.com/health/clinical-operations/systems-operations/
V15	Nervecentre: Nottingham case study	Ownership: Documented; rationale: Documented (partial)	Verified (doc review)	https://nervecentresoftware.com/wp-content/uploads/2025/04/NC0241_Nottingham-Case-Study-ARTWORK.pdf
V16	Nervecentre: NUH NeT case study	Outcome: Not documented	Verified (doc review)	https://nervecentresoftware.com/wp-content/uploads/2026/09/NUH-NeT-Case-Study.pdf
V17	Nervecentre: HSE selects for AFIS	Irish national incumbent	Verified (vendor)	https://nervecentresoftware.com/hse-selects-nervecentre-for-irelands-acute-floor-information-system/